NCCP 00310 · clinician draft

Gemcitabine and carboplatin · lung cancer

> **Supportive prescription draft for review.** This is a regimen template. It is not an issued patient prescription; individual review and completion are required.

Day 1 · gemcitabine / carboplatin

Active medicines: gemcitabine + carboplatin

Emetogenic risk: high

Medicine	Dose / route	When
Aprepitant	125 mg orally Day 1; 80 mg orally Days 2–3	Before SACT on Day 1, then daily on Days 2–3
Ondansetron	16 mg orally	Day 1 Adjust for QT/individual risk.
Antiemetic dexamethasone reference	12 mg orally Day 1; 8 mg orally daily Days 2–4	Dedicated carboplatin AUC ≥4 NCIS V 8 row Reference course only: reconcile the aprepitant interaction and any other steroids into one clear prescription.
Metoclopramide	10 mg orally three times daily as prescribed	Subsequent days NCIS row does not specify an end day or PRN qualifier; prescriber/ pharmacy must specify duration and suitability.

Additional phase-specific support

- Follow one written anti-sickness prescription, including any tablets for home and rescue treatment. Day 8 gemcitabine has a separate lower-risk anti-sickness plan.Carboplatin needs a reliable kidney-function result for dosing. Report unusual bruising or bleeding promptly.
- Follow one reconciled prescription; the clinical support sheet is a reference, not an instruction to self-prescribe.

Before signing: verify allergies, current medicines, interactions, renal / hepatic function, QT risk, sedation, prior response, total steroid exposure and the rescue-medicine duration. Do not duplicate medicines from another phase.

Individual changes / rescue duration: _____
Prescriber / date: _____ Pharmacy review: _____

NCCP 00310 v8 · pNCCP 00310 supportive care; NCIS V 8 Table 1 · NCCP NCIS antiemetics V 8

Sources checked 2026-11-02 · SACTCheck v0.84.28 · Regimen-level drafts; prescriber and oncology-pharmacy verification required